

CASE MANAGEMENT HANDOUT

OPERATIONAL CONFLICT RESOLUTION: INTAKE, PLANNING, & TREATMENT

This comprehensive operational handout provides Case Managers with actionable, step-by-step methodologies to identify, manage, and resolve critical conflicts across the Intake, Case Planning, and Active Treatment phases of a Personal Injury claim.

Proper mastery of these guardrails guarantees steady file progression, structural legal compliance, and bulletproof defense frameworks against insurance mitigation strategies.

1. Intake Phase: Conflict Identification & Gatekeeping

The Intake Phase serves as the primary quality control filter for the firm. Operational bottlenecks, structural informational deficits, and legal exposure must be systematically neutralized before a file is approved for formal case opening.

A. Core Trajectory Mapping & Conflict Resolution

- **Step 1: Execute System Matrix Setup** - Open your new file profile simultaneously in the Case Management System (CMS/Casepeer) and the firm's centralized digital storage repository (Sharepoint). Input your profile name into the dedicated 'Lien Negotiator' field to assert absolute administrative ownership.
- **Step 2: Run the Mandatory Conflict Check** - Before recording detailed narrative disclosures from the client, cross-examine the primary intake data against the internal firm index. Query the full legal names of the prospective client, all adverse operators, parent commercial entities (e.g., Apex Delivery Services / Apex Logistics), registered vehicle owners, and known eyewitnesses. If an exact name match or direct familial/business relationship appears inside an active, open, or historical file, instantly freeze the intake data entries. Do not log confidential facts. Check the box marked 'Escalated for Attorney/Senior Review' and route an urgent advisory notice to the handling attorney to clear ethics compliance.



B. Common Bottlenecks & Operational Fixes

- **Issue 1: Incomplete Primary Information or Missing Signatures** - The prospective client submits a vague narrative description of the Motor Vehicle Accident (MVA) and fails to execute authorization forms or electronic retainers.

Operational Fix: Do not let the file languish in an indeterminate status. Deploy an automated 3-day notification sequence through the CMS. If the client remains unresponsive to clarification requests after 72 hours, execute a structured verification follow-up call. If contact fails, change the file status to 'Declined due to Eligibility/Risk Factors' to protect firm resources.

- **Issue 2: Ambiguous Liability Profiles** - The initial facts of loss reveal an intersection collision where the right-of-way assignment is unclear, or the police report has been delayed by municipal agencies.

Operational Fix: Do not make unilateral case-opening decisions on unconfirmed liabilities. Categorize the profile as 'Escalated for Attorney/Senior Review'. Task a dedicated assistant to track down the unredacted accident exchange details, and preserve a clean internal case log detailing the precise open liability questions requiring legal review.

2. Case Planning Phase: Coverage & Liability Projections

Every newly accepted case file must undergo a comprehensive tri-panel assessment within the first 30 days to determine its maximum valuation and operational path.

A. The Regulatory Triple-Check

- **Step 1: HIPAA Compliance & Specialized Authorizations** - Secure an explicit, claim-specific Authorization to Release Medical Information signed by the client. The form must specifically include the exact Date of Loss (DOL) and grant the firm permission to interact with legal representatives, medical billing clearinghouses, and first-party auto insurance adjusters.

- **Step 2: Medicare/Medicaid Federal Subrogation Disclosure** - Analyze the intake demographics to confirm if the client is aged 65 or older, holds active Medicare benefits, or receives Social Security Disability Insurance (SSDI). If federal health parameters apply, you must notify the Centers for Medicare & Medicaid Services (CMS) or the Benefits Coordination & Recovery Center (BCRC) immediately. Failure to establish an official subrogation case profile early can result in permanent settlement freezes and structural federal penalties.



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- **Step 3: Map the Primary Payor Order Hierarchy** - Establish the exact coordination of benefits sequence before authorizing clinical evaluations. The standard structural payment roadmap must follow this strict tier: Tier 1: Personal Injury Protection (PIP) / MedPay -> Tier 2: Private Health Insurance / ERISA Plan -> Tier 3: Letter of Protection (LOP) / Medical Liens.

B. Analyzing Commercial & High-Value Claims (e.g., John Doe v. Apex)

When dealing with a commercial corporate defendant, your case plan must adjust to handle a complex corporate structure:

- **Coverage Limits:** Commercial carriers routinely maintain primary casualty policies starting at \$1,000,000, augmented by secondary commercial umbrella policies. Because the coverage ceiling is high, you must secure full policy limit disclosures (including declarations pages and affidavits of additional insureds) early to build accurate valuation expectations.
- **Liability Presumptions:** Rear-end collisions involving commercial trucks create a strong legal presumption of negligence on the adverse operator (e.g., following too closely, mechanical failure, or driver fatigue). However, adjusters will aggressively search for comparative negligence elements to mitigate their exposure.
- **The Action Plan:** You cannot project a case accurately without validating liability. Order the official police report immediately. If liability is contested or the policy boundaries are obscured, escalate the file directly to the litigation tracking division to initiate a formal lawsuit instead of wasting the client's treatment timeline on uncooperative adjusters.

3. Treatment Phase: Monitoring Gaps, Red Flags, & Crises

The Treatment Phase is the operational core of case management. Your primary focus is maintaining structural file integrity to defeat insurance utilization reviews.

A. Active Treatment Gap Intervention Protocols

Insurance defense software scans medical packages to identify breaks in treatment timelines. These gaps are used to argue that the client's injuries are either resolved or unrelated to the accident. Common causes include missed clinical appointments, sudden transportation failures, or personal crises.

- **Assess:** Monitor weekly provider attendance reports. If a client misses consecutive appointments or displays a 14-day gap in active care, flag the file immediately.



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- **Clarify:** Contact the client directly. Conduct a structured interview to discover the root cause of the non-compliance.
- **Track:** Record the client's specific explanation and your corrective actions into the CMS notes using formal documentation methods. This preserves an unalterable audit trail that your legal team can use to neutralize defense challenges during negotiations.

B. Case Manager Master Escalation Ladder

When an administrative, clinical, or insurance obstacle blocks the standard treatment roadmap, apply this progressive framework:

- **Level 1 (Clinical):** Contact the provider's office manager directly to secure delayed treatment records or coordinate an expedited specialist consultation.
- **Level 2 (Administrative):** Issue an internal 'Notice of Delay' to document external care barriers (such as delayed insurer authorizations for an MRI scan).
- **Level 3 (Legal):** Request the handling attorney to file a formal motion or execute an explicit Letter of Protection (LOP) to bypass insurance stalling tactics completely.

4. Clinical Simulation Workbook

Review these two common real-world operational scenarios to master advanced case tracking techniques.

CASE STUDY 1: The 'Eggshell Plaintiff' Core Strategy

- **The Conflict:** Your client, John Doe, suffered an acute L4-L5 disc protrusion following a rear-end collision by an Apex Logistics truck. The adverse carrier (Aggressive Casualty) denies authorization for an MRI. They discovered that John had a minor lumbar strain in 2018 and claim his current pain is entirely due to a pre-existing degenerative condition.

- **The Operational Solution:** You must apply the Structural Aggravation Standard. Do not allow the adjuster to label this a simple soft-tissue case. Use the following step-by-step procedure:

Step 1: Secure all pre-accident medical notes and diagnostic reports from the historical 2018 file to build a clear clinical baseline.

Step 2: Demand a post-accident MRI scan immediately. If the carrier stalls, bypass them by routing the client under a firm LOP.



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Step 3: Compare the pre- and post-accident findings side-by-side. If the 2018 report notes a stable 2mm bulge and the current MRI shows an acute 5mm herniation with nerve impingement, you have objective proof of structural aggravation.

Step 4: Frame the final demand around this objective advancement. Remind the adjuster that under the Eggshell Plaintiff Doctrine, the defendant is entirely liable for accelerating an asymptomatic condition into a severe, symptomatic state.

CASE STUDY 2: The 'Transportation Wall' Crisis Resolution

- **The Scenario:** Your client has been highly compliant for 4 weeks through an intensive 12-week chiropractic program. Suddenly, he calls you in a panic: his car's transmission just died. He lives 45 minutes away from the clinic, cannot afford a rental car, and says, 'I can't get to my appointments anymore. I might as well just quit treatment and try again next year when I have money saved.'

- **The Operational Risk:** If the client stops treatment now, a massive treatment gap will form on his file. The insurance company will argue that his injuries have resolved, destroying the settlement value of the claim.

- **Step 1: De-escalate and Educate** Validate the client's stress but explain the legal risk of stopping care. Inform him clearly that stopping treatment will freeze his medical support and allow the insurer to deny the severity of his injuries.

- **Step 2: Coordinate Alternative Transport Solutions** Check if the client's first-party automobile policy includes active MedPay or PIP transportation benefits that cover rideshare or medical transport services. Contact the treating clinic's billing office; many high-volume personal injury clinics offer complementary shuttle pick-up programs for compliant patients. If local transport options are unavailable, check if the clinic can arrange a temporary transition to home-based physical therapy or a remote telehealth monitoring program to keep the treatment timeline active.

- **Step 3: Implement an LOP Funding Pivot** If alternative transportation is unavailable, consult with the handling attorney to see if a portion of the case budget can be allocated toward a medical transport account through a letter of protection with a specialized funding company.

- **Step 4: Record the Modification** Log the exact date of the transportation failure, the client's explanation, and your corrective steps into the CMS notes. This ensures that any brief, minor disruption in care is clearly documented as an asset issue rather than a recovery or clinical plateau.



5. Revenue & Monitoring Compliance Checklist

Before marking any case file as demand-ready, you must verify that all firm cost records are completely balanced:

Pre-Litigation Investigation Fee Schedule (Add123 Platform)

Comprehensive Driver's License History Query: \$3.42 (Historical archive rate prior to 3/14/24: \$1.89)

Registered VIN / Title History Search: \$3.42 (Historical archive rate prior to 3/14/24: \$1.74)

Tag and License Plate Registration Search: \$1.74

Real-Time State Insurance Coverage Query: \$3.00

Certified Outbound Document Postal Schedules

Standard First Class Postal Rate: \$0.69 (Historical Jan 2024 scale: \$0.64)

Secure Priority Delivery Service: \$8.50

Certified Mail with Return Receipt Requested: \$8.69

Priority Mail with Signature Confirmation (Settlement Checks Only): \$12.20 (Historical Jan 2024 scale: \$12.00)

Clinical Monitoring Standards & 'Red Flag' Offer Caps

Diagnostic Category	Objective Monitoring Metric	Maximum Settlement Offer Cap
Chiropractic Care	Documented progress on the Oswestry Disability Index (ODI) or Visual Analog Scale (VAS) within 6 weeks.	\$2,500 Max Offer if PIP paid \$8k+; Full Waiver requested if PIP hit the \$10k cap (Max offer capped at \$1,000 if waiver is denied).
MRI Scan Views	Standalone facility imaging interpretations and matching independent billing ledgers.	\$800 - \$1,000 per standard view (Brain MRI capped at \$1,100) assuming zero PIP coverage. Capped at \$225 if primary PIP fee rules apply.



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Diagnostic Category	Objective Monitoring Metric	Maximum Settlement Offer Cap
Pain Management Injections	Operative reports, anesthesia charts, and device/implant invoices.	Flat \$1,500 maximum cap per procedural date for Epidural Steroid Injections, Facet Blocks, or MBBs, assuming zero PIP coverage.
Specialist Consults	Documented Date of Loss (DOL) confirmation on specialist medical reports.	Initial Orthopedic MD evaluation capped at \$300; subsequent follow-up clinical evaluations limited to \$150.